Comprehensive Critical Illness Plan

Policy Wording and Schedule of Benefits

Issued by Meridian Mutual Health Insurance (fictional). Plan code CRI.

This document is a fully synthetic specimen created for the ClaimShield AI demonstration. It does not describe a real insurance product. Words with capital initials have the meanings given in Part 1.

Read this wording together with the Schedule of Benefits. Coverage decisions are based on the clauses in Parts 1 to 8. Each clause is numbered by Section and letter, for example Clause 2.2(A).

PART 1 - SCHEDULE OF BENEFITS AND DEFINITIONS

Section 1.1 - Schedule of Benefits

Clause 1.1(A): The Annual Limit of Liability per Policy Year is USD 250,000. All benefits payable under this Plan, other than a lump-sum Critical Illness Benefit where applicable, are included within this limit.

Clause 1.1(B): The Annual Deductible is USD 250 per insured person per Policy Year. The Annual Deductible is waived for Emergency Admissions.

Clause 1.1(C): After the deductible, the Member pays 0% of Eligible Charges for Inpatient Treatment and the Insurer pays the balance.

Clause 1.1(D): The Monthly Premium for this Plan is USD 389 per Policy Month.

Section 1.2 - Key Definitions

Clause 1.2(A): Medically Necessary means health care services that a licensed physician, exercising prudent clinical judgment, would provide to a patient for the purpose of evaluating, diagnosing or treating an illness or injury, consistent with generally accepted standards of medical practice.

Clause 1.2(B): Emergency Admission means an unplanned Inpatient Treatment that begins within 24 hours of the onset of acute symptoms and is certified as urgent by the treating physician.

Clause 1.2(C): Eligible Charges means Reasonable and Customary charges for Covered Services that are not listed in Section 7.2 (Non-Payable Items).

Clause 1.2(D): Surgical Package means a single all-inclusive charge for a surgical procedure, covering the surgeon's fee, operating theatre use, standard operating-room supplies and routine post-operative care for the stay.

PART 2 - INPATIENT HOSPITALIZATION

Section 2.1 - Room and Board

Clause 2.1(A): Room and board is covered up to a maximum of USD 600 per day for each day of Inpatient Treatment.

Clause 2.1(B): Where the room charged exceeds the daily maximum in Clause 2.1(A), the excess is the responsibility of the Member and is not reimbursable. Upgrades to a higher room category are treated under Section 7.2.

Section 2.2 - Intensive Care Unit (ICU) Benefits

Clause 2.2(A): Intensive Care Unit (ICU) charges are covered up to a daily sub-limit of USD 2,000 per day, for as many days as the ICU stay is Medically Necessary.

Clause 2.2(B): ICU admission requires a physician's written statement of medical necessity, including the diagnosis code (ICD-10) and the relevant clinical findings.

Section 2.3 - Surgery and Professional Fees

Clause 2.3(A): Surgical procedures billed as a Surgical Package are payable as a single charge. Standard operating-room supplies, surgical trays and routine dressings are included in the Surgical Package and shall not be billed separately.

Clause 2.3(B): Anesthesia, intensivist and attending-physician fees are payable as Eligible Charges when supported by an itemized statement.

Clause 2.3(C): The same procedure code billed more than once on the same date of service is payable only once, unless the clinical record documents a separate and distinct procedure.

PART 3 - MEDICAL NECESSITY AND UTILIZATION REVIEW

Section 3.1 - Acute Infectious Disease & Platelet Threshold Criteria

Clause 3.1(A): Inpatient and ICU treatment for severe dengue (ICD-10 A90-A91) and similar acute infectious diseases is deemed Medically Necessary where the attending physician documents a platelet count below 20,000 per microliter (mcL) or a hematocrit rise exceeding 20% above baseline.

Clause 3.1(B): To rely on Clause 3.1(A), the Member or provider must submit the laboratory values and the attending physician's statement as part of the claim or appeal.

Clause 3.1(C): ICU admission that meets the criteria in Clause 3.1(A) is payable up to the ICU sub-limit in Clause 2.2(A) without further pre-authorization.

Section 3.2 - Utilization Review and Human Review Mandate

Clause 3.2(A): Any decision to deny a claim on the ground that treatment was not Medically Necessary must be made after individualized review of the patient's clinical record by a licensed physician. Denials based solely on automated or group-level criteria are not permitted under this Plan.

Clause 3.2(B): The Insurer shall state the specific clinical reason and the policy clause relied upon in every denial notice.

PART 4 - OUTPATIENT, DIAGNOSTICS AND PHARMACY

Section 4.1 - Outpatient Care and Diagnostic Imaging

Clause 4.1(A): Outpatient consultations are payable after a Member co-payment of 10% of the Reasonable and Customary charge.

Clause 4.1(B): Diagnostic imaging, including MRI, CT and PET scans, is payable after a Member co-payment of 10% when ordered by a physician.

Clause 4.1(C): Post-operative physiotherapy and rehabilitation are payable as outpatient services, subject to the co-payment in Clause 4.1(A).

Section 4.2 - Pharmacy and Prescriptions

Clause 4.2(A): Prescription medication is payable after a Member co-payment of 10% when prescribed by a licensed physician.

Clause 4.2(B): Over-the-counter products, vitamins and nutritional supplements are not covered.

PART 5 - CRITICAL ILLNESS, EMERGENCY AND CROSS-BORDER CARE

Section 5.1 - Critical Illness Benefit

Clause 5.1(A): A lump-sum Critical Illness Benefit of USD 100,000 is payable upon first diagnosis or surgical treatment of a Covered Critical Illness, being cancer, myocardial infarction, coronary artery bypass surgery, stroke or major organ transplant.

Clause 5.1(B): The Critical Illness Benefit is payable once per Covered Critical Illness, is in addition to the Annual Limit, and requires survival for 30 days after diagnosis.

Section 5.2 - Emergency Ambulance

Clause 5.2(A): Emergency ground ambulance transport is covered up to USD 1,000 per event within the Home Region.

Clause 5.2(B): Non-emergency patient transfers require prior approval.

Section 5.3 - Cross-Border and Travel Medical Cover

Clause 5.3(A): Emergency medical treatment outside the Home Region is covered up to USD 50,000 per Policy Year, including medical evacuation and repatriation to the Home Region.

Clause 5.3(B): Planned treatment abroad and medical tourism are not covered.

PART 6 - WAITING PERIODS AND PRE-EXISTING CONDITIONS

Section 6.1 - Initial Waiting Period

Clause 6.1(A): A waiting period of 90 days applies from the Policy Start Date to all claims, except claims arising from Accidental Injury.

Clause 6.1(B): No benefit is payable for any illness first diagnosed during the waiting period in Clause 6.1(A).

Section 6.2 - Pre-Existing Conditions

Clause 6.2(A): Pre-Existing Conditions are not covered until 24 months of continuous coverage have elapsed.

Clause 6.2(B): Pre-Existing Condition means any illness, injury or symptom for which medical advice was sought or treatment received before the Policy Start Date.

PART 7 - EXCLUSIONS AND NON-PAYABLE ITEMS

Section 7.1 - General Exclusions

Clause 7.1(A): The Plan does not cover the following: cosmetic or aesthetic surgery; dental treatment other than for accidental injury; fertility and assisted reproduction; self-inflicted injury; war and nuclear risks; experimental treatment; and hazardous professional sports.

Clause 7.1(B): Treatment, drugs or procedures that are not Medically Necessary are excluded.

Section 7.2 - Non-Payable Items

Clause 7.2(A): The following items are Non-Payable Items and are excluded from Eligible Charges: admission kit and administrative fees; room upgrade and suite surcharges; personal comfort and toiletry items; television, telephone and attendant meals.

Clause 7.2(B): A Non-Payable Item on an otherwise valid claim does not invalidate the claim; the Insurer shall exclude only the Non-Payable Item.

PART 8 - CLAIMS PROCEDURE, APPEALS AND PAYMENT

Section 8.1 - Claim Documentation

Clause 8.1(A): Every inpatient claim must include the itemized hospital statement, the ICD-10 diagnosis code, and the attending physician's discharge summary with a statement of medical necessity.

Clause 8.1(B): Claims must be filed within 90 days of discharge.

Section 8.2 - Appeals and Dispute Resolution

Clause 8.2(A): A Member may appeal a denial within 180 days. The Insurer shall decide an appeal within 30 days and shall assign a physician who was not involved in the original decision.

Clause 8.2(B): An appeal should include the denial reference code, the policy clauses relied upon and supporting clinical documentation.

Section 8.3 - Payment of Benefits

Clause 8.3(A): Approved benefits are paid only to the Member's registered account on file. Any change of payout destination requires written Member authorization verified by the Insurer.